

12 WEEKS

ADVANCED LEVEL

FULL PCT INCLUDED

SUPPLEMENT STACK

ADVANCED CUTTING STEROID CYCLE

12-Week Complete Protocol — Dosages · AI · PCT · Supplements

AMAN TRAINING CLUB • Professional Series • 2024 Edition

AMAN TRAINING CLUB

■ **MEDICAL DISCLAIMER:** This document is for informational and educational purposes only. Anabolic steroids are controlled substances in many countries. Always consult a licensed physician before beginning any hormonal protocol. Misuse may cause serious health risks.

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1. OVERVIEW & GOALS

This advanced 12-week cutting protocol is designed for experienced athletes (minimum 3+ years of natural training + prior steroid experience) who want to achieve maximum fat loss while preserving — or even gaining — lean muscle mass. The stack prioritizes **hardness**, **vascularity**, and **definition** while keeping water retention minimal.

- Aggressive fat loss (target: -8 to -12 lbs body fat over 12 weeks)
- Maintain or increase lean muscle mass
- Improve muscle hardness, definition, and vascularity
- Boost strength despite caloric deficit
- Minimize water retention and estrogen-related side effects

2. CORE COMPOUND STACK

Compound	Short Name	Dose	Frequency	Duration	Purpose
Testosterone Propionate	Test Prop	100–150 mg	EOD	Wks 1–12	Androgenic base
Trenbolone Acetate	Tren Ace	75–100 mg	EOD	Wks 1–12	Hardening + fat loss
Masteron Propionate	Mast Prop	100 mg	EOD	Wks 1–12	Anti-estrogen + hardness
Primobolan	Primo	600 mg	Weekly	Wks 1–12	Lean muscle + low sides
Anavar (Oxandrolone)	Var	50 mg/day	Daily	Wks 1–8	Strength + hardness
Winstrol (Stanozolol)	Winny	50 mg/day	Daily	Wks 9–12	Final hardening

3. 12-WEEK DOSAGE SCHEDULE

Week	Test Prop (EOD)	Tren Ace (EOD)	Mast Prop (EOD)	Primo (Weekly)	Anavar (Daily)	Winny (Daily)
1	100 mg	75 mg	100 mg	600 mg	50 mg	—
2	100 mg	75 mg	100 mg	600 mg	50 mg	—
3	100 mg	75 mg	100 mg	600 mg	50 mg	—
4	150 mg	100 mg	100 mg	600 mg	50 mg	—
5	150 mg	100 mg	100 mg	600 mg	50 mg	—
6	150 mg	100 mg	100 mg	600 mg	50 mg	—
7	150 mg	100 mg	100 mg	600 mg	50 mg	—
8	150 mg	100 mg	100 mg	600 mg	50 mg	—
9	150 mg	100 mg	100 mg	600 mg	—	50 mg
10	150 mg	100 mg	100 mg	600 mg	—	50 mg
11	150 mg	100 mg	100 mg	600 mg	—	50 mg
12	150 mg	100 mg	100 mg	600 mg	—	50 mg

4. AROMATASE INHIBITOR (AI) PROTOCOL

- **Arimidex (Anastrozole):** 0.5 mg every other day (EOD) throughout the cycle
- **Cabergoline (Caber):** 0.25 mg twice per week to control prolactin (from Tren)
- Monitor estrogen via blood work; adjust AI dose based on E2 levels
- Target estrogen range: 20–30 pg/mL for optimal fat loss and libido
- Do NOT crush E2 to zero — leads to joint pain, low libido, and mood crashes

■ 5. CLENBUTEROL & T3 CYCLE

Clenbuterol Protocol (2 weeks on / 2 weeks off):

- Start: 20 mcg/day → increase by 20 mcg every 2 days
- Max: 100–120 mcg/day (women: 80 mcg max)
- Always taper down over last 2 days of each ON cycle
- Side effects: tremors, insomnia, elevated heart rate — reduce dose if severe

T3 (Cytomel) Protocol:

- Start: 25 mcg/day — Week 1–2
- Increase: 50 mcg/day — Week 3–10
- Taper: 25 mcg/day — Week 11–12
- ■ Never exceed 75 mcg/day; always taper, never stop cold turkey
- Monitor thyroid via blood work before and after cycle

■ 6. HGH FRAGMENT 176-191 & IGF-1 LR3

- **HGH Fragment 176-191:** 500 mcg/day — split into 2 injections (morning + pre-workout)
- Fasted injection protocol provides best fat-burning results
- **IGF-1 LR3:** 40–60 mcg post-workout on training days only
- Maximum cycle length for IGF-1 LR3: 4–6 weeks (receptor desensitization)
- Reconstitute with bacteriostatic water; store refrigerated

■ 7. MENT — OPTIONAL ADD-ON

MENT (Trestolone Acetate) is an extremely potent compound, roughly 10× the anabolic activity of testosterone. It is **optional** and only recommended for highly advanced users.

- Dose: 25 mg/day (injected daily or EOD)
- Duration: Weeks 1–6 only
- MENT aromatizes heavily — increase AI dose significantly if added
- Monitor blood work every 3 weeks if using MENT
- Not recommended for first-time or intermediate users

■ 8. POST CYCLE THERAPY (PCT) — WEEKS 13–16

PCT begins **3 days after last short-ester injection** (Test Prop / Tren Ace / Mast Prop). If using Primobolan alone (long ester), begin PCT 14 days after last injection.

Compound	Dose	Frequency	Duration	Purpose
HCG	500 IU	Every 3 days	Wks 13–14	Kick-start natural testosterone
Clomid (Clomiphene)	50 mg/day	Daily	Wks 13–16	LH/FSH recovery
Nolvadex (Tamoxifen)	20 mg/day	Daily	Wks 13–16	SERM estrogen control
Arimidex	0.25 mg	EOD	Wks 13–15	Gradual AI taper

■ 9. ESSENTIAL SUPPLEMENT STACK

Liver Support:

- Milk Thistle (Silymarin) — 600–800 mg/day
- TUDCA (Tauroursodeoxycholic Acid) — 500 mg/day (especially with oral steroids)

Cardiovascular Support:

- Omega-3 Fish Oil — 4–6 g/day (EPA+DHA)
- CoQ10 (Ubiquinol) — 200 mg/day
- Red Yeast Rice — 1200 mg/day (cholesterol management)
- Hawthorn Berry Extract — 500 mg twice daily

Joint Support:

- Glucosamine Sulphate — 1500 mg/day
- Chondroitin Sulphate — 1200 mg/day
- Collagen Peptides — 10 g/day

Hormonal Balance:

- DIM (Diindolylmethane) — 200 mg/day (estrogen metabolism)
- ZMA (Zinc + Magnesium + B6) — nightly before bed

Urinary/Prostate Health:

- Cranberry Extract — 400 mg/day
- Saw Palmetto — 320 mg/day

General Health:

- High-quality Multivitamin — daily with meals
- Vitamin D3 + K2 — 5000 IU D3 / 100 mcg K2
- Electrolytes — daily (especially with Clenbuterol/T3)
- Whey Protein — 40–60 g/day around training
- BCAAs — 10 g intra-workout

■ 10. TRAINING & NUTRITION GUIDELINES

Training Split:

- 5–6 days/week: Push / Pull / Legs split with 1–2 rest days
- Cardio: 30–45 min moderate intensity (LISS) 4–5×/week, fasted preferred
- HIIT: 2×/week max — avoid overtraining in caloric deficit
- Prioritize compound movements; add isolation for detail/separation

Nutrition:

- Caloric deficit: 300–500 kcal below TDEE
- Protein: 1.5–2.0 g/lb lean body mass (minimum 200 g/day)
- Carbohydrates: Timed around workouts; reduce on rest days
- Fats: 20–25% of total calories (healthy sources only)
- Water: Minimum 4 litres/day — increases with Clenbuterol use

■ 11. BLOOD WORK & HEALTH MONITORING

- **Pre-cycle:** Full CBC, testosterone (total + free), LH, FSH, estradiol (E2), liver panel (ALT/AST), lipid panel, kidney function (creatinine, eGFR), thyroid (TSH, T3, T4)
- **Mid-cycle (Week 6):** Testosterone, E2, liver panel, lipid panel, hematocrit
- **End of cycle (Week 12):** Full panel same as pre-cycle
- **PCT Week 4:** Testosterone, LH, FSH, E2 — assess recovery
- If ALT/AST >3× upper normal: stop all orals immediately
- If hematocrit >52%: donate blood or reduce dosage

■ *This guide is provided for educational purposes only. The use of anabolic steroids without a prescription is illegal in many jurisdictions and carries significant health risks. AMAN TRAINING CLUB does not endorse illegal activity. Always consult a medical professional.*